

## **PEST CONTROL**

**MCP: FAC-010**

VA Black Hills Health Care System  
Fort Meade, SD 57741  
Hot Springs, SD 57747

**Rescinded Document**

See paragraph 7

**Effective Date:**

July 7, 2022

**Signatory Authority**

VA Black Hills Health Care System  
Director

**Recertification Date:**

July 31, 2027

**Responsible Owner:**

Chief, Facilities Management

### **1. POLICY**

- a. It is the policy of VA Black Hills Health Care System to assure that the principles of an Integrated Pest Management Program are carried out for the protection of patients, staff and visitors, and to protect against damage to buildings and infrastructure including utility systems.
- b. Integrated Pest Management (IPM) is a process for achieving long term, environmentally sound pest suppression using a wide variety of technological and management practices. Control strategies in an IPM Program extend beyond the application of pesticides to include structural and procedural modifications that reduce the food, water, harborage, and access used by pests.
- c. VA BHHCS will comply with the Green Environment Management Systems (GEMS); Federal Insecticide Fungicide Rodenticide Act (FIFRA); Environmental Protection Agency (EPA); and Occupational Safety and Health Administration (OSHA) regulations as they apply to Integrated Pest Management.

### **2. JUSTIFICATION**

This policy provides the requirements for establishing and maintaining an Integrated Pest Management Program (IPM), which promotes safe, efficient, and environmentally preferred strategies, and prevents or controls disease vectors and other pests that may adversely affect health, impede operations, or damage property. Additionally, it provides the responsibilities for addressing bedbug issues within VA Black Hills Health Care System.

### **3. RESPONSIBILITIES**

- a. **The Environmental Program Manager/Assistant Program Manager, Facilities Management Service** is responsible for the administration of the Pest Management Program for the Medical Centers and ensures an Integrated Pest Management Program is accomplished and documented properly.
- b. **State licensed pest control applicators** will ensure all guidelines and procedures are followed in compliance with all Federal, State and local laws **VA**
- c. **Employees will assist the Integrated Pest management Program By**
  - (1) Maintaining a state of physical and biological cleanliness of the medical center
  - (2) Refraining from storing of food in work areas. If storage occurs, all food items must be stored in sealable plastic containers
  - (3) Assuring that break rooms, office coffee makers and dishes are cleaned daily
  - (4) Instructing patients about their responsibility regarding storage and disposition of food. Patients are discouraged from storing food in bedside lockers. If storage occurs, all food items must be stored in sealable plastic containers. Regular preventative inspections will be conducted by area staff. Environmental Management Section will provide advisement and assistance as necessary.

#### 4. PROCEDURES

- a. All areas of the Medical Center will be routinely inspected for pest infestations and pest prevention measures. At a minimum all food service areas will be inspected monthly, operating rooms (OR) and supply processing and distribution (SPD) areas will be inspected quarterly, patient treatment areas semi-annually, and all other areas annually. Any problems identified in between inspections will be handled immediately as appropriate.
- b. Conduct inspections to determine which pest management measures are appropriate, required, and effective. Identify target pests and their route of entry into buildings.
- c. Recommend environmental sanitation practices that restrict or eliminate food, water, or harborage for pests. Removing supplies from cardboard boxes rather than storing cardboard in work areas, storing supplies off the floor utilizing pallets and cleaning out unused items in cabinets and storerooms.
- d. Utilizing basic methods (caulking, sealing cracks, etc.) for exclusion of pests. Keeping all windows closed unless screens are provided and promptly closing doors behind traffic (do not "prop" doors open). Reporting to Engineering areas requiring repairs, windows lacking screens, water leaks (no matter how small), or

improvement needed to prevent the entry of pests into the Medical Center or surrounding buildings.

- e. Selection and utilization of non-chemical control methods to the greatest extent possible that eliminates, excludes or repels pests (i.e., insect electrocution devices, traps, air screens, etc.).
- f. Selection and use of the most environmentally sound pesticide(s) to affect control when chemical control methods are necessary. All pesticides used at this facility are registered with the US Environmental Protection Agency (EPA). SDS's are located online.
- g. All pesticides in the health care environment will be applied by a state licensed pest controller. No pesticide chemicals will be applied in any "Clean Room", such as, the OR and SPS areas. No personally purchased over-the-counter pesticides may be brought into the Medical Center. Quarters residents are the only exception; they may purchase and apply in their own living areas.
- h. The applications of herbicide and pesticide chemicals to station grounds will be the responsibility of the Engineering Program Manager, Facilities Management Service.
- i. The harboring and feeding of wildlife (ducks, squirrels, and feral cats) is prohibited throughout the Medical Center. Recreation Therapy is authorized the use of bird feeders in long term care areas. Recreation Therapy is responsible for storage of bird seed in sealable containers and the cleanup of seed husks in those areas.
- j. Prevention of Bedbugs:
  - (1) VA BHHCS has established a systematic approach to prevent and control any potential bed bug infestation for inpatient areas, outpatient areas, and transitional recovery program areas. Prevention is focused on avoiding the transfer of bed bugs to the facility and reducing potential places they harbor.
  - (2) It is important for all health care providers to recognize the signs and/or symptoms in patients caused by bed bugs in order to provide appropriate care and to be alerted to possible infestation. Health conditions associated with bed bugs include bed bug bites, usually clustered closely together in a line on the arms, shoulders, neck and legs.
  - (3) Identify areas that may be a particularly high risk and susceptible to an infestation, such as any residential programs, e.g., community living centers,

inpatient psychiatric services, domiciliary programs, where patients arrive for care with their own luggage or belongings. Health Technicians should encourage patients to:

- (a) Allow them to visually inspect their belongings.
  - (b) Shower immediately upon admission.
  - (c) Prior to transferring their personal belongings into their living space, they should wash all clothing and dry at high heat for a minimum of one hour. This may require the Health Technicians to schedule the washers and dryers immediately. Some judgment needs to be applied in the event some personal belongings can't withstand the high heat.
  - (d) Place work orders to seal any cracks and crevices in the room and repair any torn wallpaper.
  - (e) Inspect bed linens for dark spots and bugs, and to notify the EMS office immediately if there is any suspicion of bed bug infestation.
  - (f) Ensure that discharge cleaning is performed prior to assigning any new patient to that living area. During any discharge cleaning the room should be inspected for any signs of bed bug infestation, e.g., inspecting the mattress, replacing any mattress that have tears, inspecting bed boards if they are present, and inspecting dressers and/or closets.
- (4) Avoid the use of box springs since bed bugs can easily hide inside; however, if this is not possible the box springs should be placed inside a complete cover.
  - (5) Avoid placing beds in direct contact with walls.
  - (6) Avoid the use of headboards that are attached to the wall.
  - (7) Avoid leaving any excess belongings under or on the floors. This provides a place for bed bugs and other pests to hide. Excessive belongings can be placed/stored in the patient baggage room.
  - (8) If it is suspected that a patient's living area has bed bugs the following actions should be taken immediately.

- (a) If the patient is still available, they should be instructed to shower and provided a pair of hospital pajamas to wear temporarily.
- (b) All the patient's belongings should be bagged (except toiletries) and sealed until they can be washed and dried on high heat for a minimum of one hour. For items that can't be laundered they should remain in a sealed bag until a professional pest controller can inspect the items.
- (c) The patients living area should be closed off and no items should be removed except for those that have been placed into a sealed plastic bag. Removing any items can raise the risk of spreading the bed bug infestation to other areas of the facility. Avoid entering the room until a professional pest controller can inspect and treat the room if necessary. Do not reassign the room to any patient until the room has been declared free from bed bug infestation.
- (d) Collect any live or dead bugs, if possible, for inspection by a professional pest controller for confirmation.
- (e) Notify the EMS department immediately.
- (f) If possible, discuss information on bed bug detection, prevention, and control with the patient.
- (g) If an outpatient is discovered to have bed bugs the following actions should be taken immediately.
  - 1) Provide a plastic bag and patient pajamas to the patient so that they can change into the pajamas and place their belongings into the plastic bag.
  - 2) Seal the bag and instruct the patient to wash their belongings immediately upon the return home and dry on high heat for a minimum of one hour.
  - 3) If possible, do not allow the patient to travel to any other places within the facility until they have changed into the pajamas and sealed their belongings in the plastic bag. Notify the EMS department immediately upon discovery of the potential bed bug infestation.
  - 4) Take the room that the patient was in out of service until it can be inspected and possibly treated by a professional pest controller.

- 5) If it can be determined that the patient traveled anywhere else within the facility prior to the actual discovery those areas should be shut down and inspected and possibly treated also.
- 6) Bed bugs are nocturnal and the known hiding areas would be underneath the seat of chairs and/or in cracks and crevices
- 7) If possible, discuss information on bed bug detection, prevention, and control with the patient.

## 5. DEFINITIONS

Bedbugs. Bedbugs are small, brownish, flattened insects that feed on blood. The main source for blood meals (i.e., “bedbug bites”) is humans, although other mammals and birds can serve as a source. The common bedbug, *Cimex lectularius*, is the species most adapted to living with humans. They are nocturnal and typically bite people while they sleep. No building structure or common source of transportation is immune to bedbug infestation. Bedbugs commonly hide in mattresses, carpets, behind electrical plates, peeling paint or wallpaper, electronics, and in furniture crevices. Although bedbugs can harbor pathogens in their bodies, there are no known documented cases of transmission to humans.

## 6. REFERENCES

- a. VHA Program Guide 1850.2 Pest Management Operations  
Pest [Control](#) 1850.2
- b. EPS Bed Bug Management Guide  
<http://vaww.vhaco.va.gov/EPS/Directives/Bed%20Bug%20Guide2.pdf>
- c. Environmental Protection Agency. “Pesticide Registration (PR Notice) Notice 2002-1” Pesticide Registration (PR) Notices. 2002.  
[http://www.epa.gov/PR\\_Notices/pr2002-1.pdf](http://www.epa.gov/PR_Notices/pr2002-1.pdf) November 30, 2010
- d. Environmental Protection Agency. “Bed Bugs: Get Them Out and Keep Them Out” <https://www.epa.gov/bedbugs> November 30, 2010

## 7. RESCISSION

BHHCS Policy FAC-10 dated 4/21/2016

## 8. REVIEW

This MCP will be reviewed at recertification and when there are changes to the governing document.

## 9. RECERTIFICATION

This MCP is scheduled for recertification on or before the last working day of July 2027. This MCP will continue to serve as local policy until it is recertified or rescinded. In the event of contradiction with national policy, the national policy supersedes and controls.

## 10 SIGNATORY AUTHORITY

Lisa R. Curnes  
302676

Digitally signed by Lisa R.  
Curnes 302676  
Date: 2022.07.08  
14:31:56 -06'00'

Lisa R. Curnes, MHA  
Director VA Black Hills Health Care System  
**Date Approved:** July 7, 2022

**Note:** *The date approved is when the policy was approved by the owning council and replaces a physical signature. The approval remains valid until rescinded by an appropriate administrative action.*

**Distribution:** [Active MCP's are stored in the VA BHHCS Policy SharePoint](#). This policy was routed through the following collaboration and approval groups and individuals before being approved through the appropriate council and the Director: